Blast Injury / Incident

TB-01 | Trauma & Burns | Reviewed

Clinical release pending. Verify current approved protocols and standing orders.

Quick flow

01 Scene Safe? [EMT / AEMT / Paramedic]

Stage for hazards, structural collapse, fire, hazardous material, or secondary device; activate the agency MCI plan when indicated.

02 Triage + Rapid Removal [EMT / AEMT / Paramedic]

Quantify patients, request resources, apply the local MCI/triage process when indicated, and begin load-and-go care.

03 Identify Injury Pattern [EMT / AEMT / Paramedic]

Airway burn • blast lung • hemorrhage • penetrating trauma • crush; consider radiation only when the mechanism suggests a radiological or nuclear event.

04 Treat Immediate Threats [EMT / AEMT / Paramedic]

Airway and ventilation • hemorrhage control • assess chest injury • hypothermia prevention • reassessed shock care. Urgently request ALS for suspected tension pneumothorax; decompression is Paramedic-only. Ventilate when clinically necessary.

05 Blast Lung Injury? [EMT / AEMT / Paramedic]

Dyspnea, hypoxia, hemoptysis, wheeze, chest pain, or diminished breath sounds; perform serial respiratory assessment, SpO_2, lung sounds, and monitoring when available.

06 Rapid Trauma Transport [EMT / AEMT / Paramedic]

Use trauma/burn destination plan and notify early, especially for respiratory compromise, penetrating injury, major burn, crush injury, or multisystem trauma.

Overview

- Treat every blast as a potentially unsafe, intentional event until authorities determine otherwise.
- Expect combined primary, secondary, and tertiary blast injury, including barotrauma, penetrating trauma, burns, crush injury, and toxic exposure.

Provider-Level Actions

Perform all immediately indicated care within scope without delaying rapid trauma transport.

EMT

- Maintain scene awareness, perform triage, control major hemorrhage, assess chest injury, oxygenation, ventilation, and perfusion, urgently request ALS for suspected tension pneumothorax, support ventilation, and prevent hypothermia.
- For an open chest wound in a spontaneously breathing patient, consider an approved vented seal and reassess frequently. If deterioration follows sealing, assess/release an obstructing seal and obtain immediate Paramedic treatment for suspected tension pneumothorax. Do not probe the wound. Positive-pressure ventilation requires the chest-injury standing orders and immediate Paramedic/Medical Control guidance, not automatic extrapolation of this seal instruction.
- Assess for hearing loss, ocular injury, burns, penetrating injury, abdominal injury, amputation, and crush syndrome.

AEMT

- Perform all EMT care plus IV access and reassessed crystalloid only when indicated for shock after hemorrhage control, without delaying transport.
- Use authorized airway adjuncts and monitor closely for worsening blast-lung physiology.

Paramedic

- Perform all prior care plus advanced airway management, cardiac/$EtCO_2$ monitoring, and chest decompression when indicated.
- When positive-pressure ventilation is clinically necessary for suspected blast lung injury, use the lowest effective pressures/volumes and monitor closely for deterioration.

Indications

- Patient with suspected blast injury / incident requiring prehospital assessment or treatment.

Warnings / precautions

- Do not approach a patient who may be attached to wires, a package, or a triggering mechanism; withdraw and notify law enforcement.
- Do not withhold needed ventilation. Avoid unnecessarily high positive pressures/volumes and excessive fluids in suspected blast lung injury; if air transport is used, notify the flight crew of the concern.

Clinical pearls

- Tympanic membrane rupture and hearing loss are common and may impair communication.
- Secondary blast injuries from debris and shrapnel are the most common cause of death.

Source and references

Claiborne County EMS native protocol content; controlled PDF generated from the same structured source. | Current Tennessee EMS scope of practice, verified individual credentials, and Claiborne County standing orders control provider authorization.

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